FOR EDITH · PAEDIATRICS & CHILD HEALTH JUNIOR CLERKSHIP

Child Rights & Violations in Uganda

Lecture | Week 6 · Tetanus, skin, poisons & hormones | Paediatrics | Dr Namugga

Every child under 18 has rights to survival, development, protection and participation under the UN Convention on the Rights of the Child, the African Charter, Uganda's Constitution and the Children Act. A health worker must recognise abuse, neglect and defilement, keep the child safe, examine and document carefully (Police Form 3/3A), give emergency medical care (PEP, emergency contraception, STI prophylaxis) and report to the police, the Probation and Social Welfare Officer or the Sauti 116 helpline.

Must know before the ward round

1. A child is anyone under 18 years (UNCRC Art. 1, African Charter, Uganda Constitution, Children Act). The age of sexual consent in Uganda is 18, so any sexual act with a person under 18 is defilement (Penal Code Act s.129), whatever the 'consent'.
2. Aggravated defilement (s.129(3)–(4)): the victim is under 14; or the offender is HIV-positive, a parent/guardian/person in authority, or a serial offender; or the victim has a disability. Maximum penalty: death.
3. UNCRC (1989; Uganda ratified 1990): four general principles: non-discrimination (Art. 2), best interests of the child (Art. 3), right to life, survival and development (Art. 6), and respect for the child's views (Art. 12).
4. Forms of maltreatment (WHO): physical abuse, sexual abuse, emotional (psychological) abuse, neglect, and exploitation (commercial, labour, trafficking). Uganda adds child marriage, FGM, child sacrifice and harmful traditional practices.
5. Uganda VACS 2015: about 1 in 3 girls and 1 in 6 boys experienced sexual violence in childhood, and most children experienced physical violence, usually from parents, teachers or partners (VERIFY exact figures).
6. Bruising red flags (TEN-4-FACESp): bruises on the Torso, Ears or Neck in a child under 4; ANY bruise in an infant under 4 months; Frenulum, Angle of jaw, Cheek, Eyelid, Subconjunctival; Patterned bruising.
7. Fractures highly suggestive of abuse: posterior rib fractures, metaphyseal corner ('bucket-handle') fractures, multiple fractures of different ages, fractures in a non-mobile infant, complex skull fractures. Do a skeletal survey under 2 years.
8. Abusive head trauma (shaken baby): subdural haemorrhage + retinal haemorrhages + encephalopathy in an infant, with an inadequate history. Seizures, apnoea, vomiting, bulging fontanelle.
9. After sexual assault within 72 hours: HIV PEP for 28 days (UCG 2023 lists ABC + 3TC + LPV/r for children; DTG-based regimens are now preferred where available), emergency contraception (levonorgestrel 1.5 mg, within 72 h and up to 120 h; double the dose on enzyme-inducing ARVs), STI prophylaxis, hepatitis B vaccine, tetanus, pregnancy test, psychosocial support.
10. Documentation: history in the child's own words (quoted), body map, measured and described injuries, photographs with consent, chain of custody for samples; complete Police Form 3 (injuries) or Police Form 3A (sexual offences).
11. Reporting pathway: police Child and Family Protection Unit; the district Probation and Social Welfare Officer (PSWO); the LC1 chairperson; Sauti 116 child helpline (toll-free). Admit the child to hospital as a place of safety if home is unsafe.
12. Neglect is the commonest form of maltreatment: failure to thrive, missed immunisations, untreated illness, abandonment and lack of supervision; distinguish it from poverty by asking whether the carer uses the resources available.

1 In a nutshell
2 Definitions & classification
3 Epidemiology & Uganda context
4 Pathophysiology: why abuse happens and how it harms
5 Clinical acumen
6 Clinical workup
7 Differential diagnosis
8 Investigations
9 Management
10 Complications
11 Ward-round & exam pearls
12 Mnemonics & memory aids
13 High-yield summary

In a nutshell

Child rights are the entitlements every person under 18 has to **survive, develop, be protected and participate**. **Child maltreatment** (abuse and neglect) is any act or failure to act by a parent, carer or person in authority that results in actual or potential harm to the child's health, survival, development or dignity.

Maltreatment is **common in Uganda** and mostly **hidden**: in the home, in school, in the community, by people the child knows and trusts. It presents to health workers disguised as an injury, a burn, failure to thrive, a vaginal discharge, a teenage pregnancy, a behavioural problem or a child brought "late".

Health workers have three jobs:

1. **Recognise** it: think of abuse when the story doesn't fit the injury.
2. **Respond medically**: treat injuries, prevent HIV, pregnancy and STIs, and document as legal evidence.
3. **Protect**: keep the child safe and **report** to the police, the Probation and Social Welfare Officer (PSWO) or the **Sauti 116** helpline, working with social workers.

> **CLINICAL PEARL**
> The one thing never to forget: **the child is your patient, and the child's safety comes first.** If you suspect abuse and the child would go home to the abuser, admit the child (hospital as a place of safety) while you involve the PSWO and police. Never confront a suspected abuser in a way that puts the child at more risk.

Definitions & classification

Term	Meaning
Child	Any person under **18 years** (UNCRC Art. 1; African Charter Art. 2; Uganda Constitution; Children Act)
Child maltreatment (WHO)	All forms of physical and/or emotional ill-treatment, sexual abuse, neglect or negligence and commercial or other exploitation that harm, or could harm, the child's health, survival, development or dignity, in a relationship of responsibility, trust or power
Physical abuse	Intentional use of force: hitting, kicking, shaking, burning, scalding, poisoning, suffocating, excessive corporal punishment
Sexual abuse	Involving a child in sexual activity they cannot understand or consent to, or that breaks the law: contact (touching, penetration) and non-contact (exposure, pornography, grooming). In Uganda any sexual act with a person under 18 is **defilement**
Emotional abuse	Persistent rejection, threats, humiliation, terrorising, isolating, exposure to domestic violence
Neglect	Persistent failure to meet basic needs (food, shelter, hygiene, supervision, health care, education, emotional care), when the carer could reasonably do so
Exploitation	Child labour (hazardous work), commercial sexual exploitation, trafficking, use in armed conflict, begging
Harmful traditional practices	Child marriage, female genital mutilation (FGM), "ebiino" (extraction of infants' canine tooth buds), child sacrifice, witchcraft accusations, scarification
Defilement (Penal Code s.129)	Unlawful sexual act with a person under 18
Aggravated defilement (s.129(3)–(4))	Victim under 14; offender HIV-positive; offender is a parent, guardian or person in authority; offender a serial offender; or victim has a disability

The four categories of rights (UNCRC)

Category	Examples	Health worker's role
Survival	Life, health care, nutrition, clean water (Art. 6, 24)	Treat, immunise, feed
Development	Education, play, rest, freedom of thought (Art. 28, 31)	Growth and development monitoring, ECD
Protection	From violence, abuse, neglect, exploitation, harmful practices, child labour (Art. 19, 32, 34–39)	Recognise, treat, report
Participation	To be heard in decisions affecting them (Art. 12), freedom of expression	Listen to children; assent; confidentiality for adolescents

Epidemiology & Uganda context

- **WHO (global):** about **6 in 10** children under 5 regularly suffer physical punishment and/or psychological violence from caregivers; **1 in 5 women and 1 in 7 men** report being sexually abused as a child; about 40,000 child homicides a year, many misattributed to falls, burns or drowning.
- **Uganda Violence Against Children Survey (VACS, 2015):** about **1 in 3 girls (35%)** and **1 in 6 boys (17%)** experienced sexual violence in childhood; around **6 in 10 girls and 7 in 10 boys** experienced physical violence, most often by parents, caregivers and **teachers**; few children told anyone and fewer received services (VERIFY figures against the published report).
- **Child marriage and adolescent pregnancy:** about one-third of Ugandan women aged 20–24 married before 18, and about **1 in 4 girls aged 15–19** have begun childbearing (UDHS). Every pregnancy in a girl under 18 involves an offence (defilement), though prosecution depends on the circumstances.
- **Defilement** is among the most reported crimes in Uganda's annual police crime reports. Most offenders are known to the child: neighbours, relatives, teachers, boda-boda riders, "boyfriends" and peers.
- **Other Ugandan concerns:** child labour (agriculture, domestic work, mining, stone quarrying, fishing), street children (Kampala, from Karamoja), trafficking, **FGM** in Sebei and Pokot communities (banned by the Prohibition of FGM Act 2010), **child sacrifice** (rare but reported), witchcraft accusations against children with disabilities, and violence in **refugee** settings.

Ugandan legal and policy framework

Instrument	Key provisions for health workers
UN Convention on the Rights of the Child (1989)	Ratified by Uganda **1990**. Four principles; Art. 19 (protection from violence), Art. 24 (health), Art. 34 (sexual exploitation)
African Charter on the Rights and Welfare of the Child (1990)	Ratified **1994**. Child = under 18 without exception; Art. 21 bans **child marriage** and harmful practices; children have duties too
Constitution of Uganda (1995)	**Art. 34**: rights of children (to know and be cared for by parents, education, health, protection from exploitation and hazardous work); **Art. 31**: marriage only from 18
Children Act (Cap 59), amended 2016	Defines child as under 18; **welfare principle** (best interests); parental responsibility; care and protection orders; Family and Children Court; role of the **PSWO**; 2016 amendment strengthened protection from **violence, harmful practices, child labour and trafficking**, and banned **corporal punishment in schools** (VERIFY section numbers; the Act was renumbered in the 2023 revised laws)
Penal Code Act (as amended 2007)	**s.129 defilement** (under 18), **aggravated defilement** (death penalty maximum); rape; indecent assault; age of criminal responsibility **12** (Children Act). An offender under 18 is dealt with under the Children Act

Instrument	Key provisions for health workers
Prohibition of FGM Act (2010)	Criminalises FGM, including procuring or failing to report it
Domestic Violence Act (2010)	Protection orders; health workers record and refer
Prevention of Trafficking in Persons Act (2009)	Trafficking of children is aggravated
Employment Act (2006) and Employment of Children Regulations (2012)	Restricts child labour; no hazardous work under 18
National Child Policy (2020) and **National Plan of Action on VAC**	Coordinated prevention and response

UGANDA CONTEXT

The people in the protection system you will work with: the **Probation and Social Welfare Officer (PSWO)** at the district (statutory lead for child protection), the police **Child and Family Protection Unit (CFPU)**, the **LC1 chairperson**, the hospital **medical social worker**, community development officers, NGOs (e.g. ANPPCAN, Raising Voices, Plan, World Vision) and the **Sauti 116** national child helpline (free, 24 hours). Courts: the **Family and Children Court** (for care orders and children in conflict with the law) and the High Court for aggravated defilement.

Pathophysiology: why abuse happens and how it harms

Risk factors (ecological model)

Level	Risk factors
Child	Age under 4 (physical abuse) or adolescence (sexual abuse), disability, prematurity, chronic illness, crying infant, unwanted child, orphan, girl (sexual abuse, marriage), living away from parents
Caregiver	Young age, substance (alcohol) misuse, mental illness, own history of abuse, unrealistic expectations, step-parent or non-relative in the home
Family	Domestic violence, poverty, overcrowding, social isolation, family breakdown, polygamous conflicts
Community /society	Social acceptance of corporal punishment, gender inequality, conflict and displacement, weak law enforcement, "settling" defilement cases between families for money

How maltreatment harms the child

- **Physical**: injuries, disability, death; abusive head trauma kills or disables about one-third of victims.
- **Toxic stress**: chronic activation of the stress response damages the developing brain (memory, emotional regulation), immune function and growth. Adverse childhood experiences (**ACEs**) increase lifelong risk of depression, substance misuse, HIV, chronic disease and becoming a perpetrator (the **intergenerational cycle**).
- **Sexual**: STIs including HIV, pregnancy (high risk of obstructed labour, fistula, unsafe abortion and death in young adolescents), genital injury, school drop-out, PTSD.

Clinical acumen

Focused history

Take the history from the carer, then, **if the child is old enough and willing, from the child alone** (with a chaperone of the child's choosing).

Ask	Why it matters
Exactly how did the injury happen? Who saw it? When?	A vague, changing or implausible story is the key warning sign
Delay in seeking care	Delay without good reason suggests concealment
Is the mechanism compatible with the child's **developmental stage**?	"He rolled off the bed and broke his femur" in a 2-month-old who cannot roll: inconsistent
Previous injuries, admissions, burns	Pattern of repeated harm
Who lives at home and who looks after the child	Identifies possible perpetrators and protective adults
Domestic violence, alcohol, mental illness	Risk factors
Growth, feeding, immunisation, school attendance	Neglect
For sexual abuse: **disclosure in the child's own words** (record verbatim), timing (within 72 hours?), type of contact, pain, bleeding, discharge, menarche, contraception	Medical needs (PEP, EC), forensic sampling, legal evidence
Behaviour changes: withdrawal, aggression, sexualised behaviour, bed-wetting, school refusal, self-harm	Emotional and sexual abuse

> NOTE
> Let a child **disclose freely**. Use open questions ("Tell me what happened"), avoid leading questions ("Did your uncle touch you?"), do not promise to keep secrets, tell the child they did the right thing by telling you, and write down their exact words. Repeated interviews re-traumatise the child and weaken the court case: one good interview is better than many.

Examination

- Do a **full top-to-toe examination** with the child undressed in stages (keep warm, respect privacy), with a chaperone.
- **Growth**: weight, height, head circumference, Z-scores (neglect).
- **Skin**: bruises (site, size, colour, shape), burns, bites, scars of different ages. Use a **body map**; measure with a ruler.
- **Head**: scalp (bald patches from hair pulling, swellings), **fontanelle**, ears (bruising inside/behind), eyes (subconjunctival haemorrhage; **fundoscopy** for retinal haemorrhages in infants).
- **Mouth**: torn **frenulum** in an infant (force-feeding, blow), dental injuries, palatal petechiae (oral sex).
- **Skeleton**: tenderness, swelling, deformity, reduced movement.
- **Abdomen**: bruising, tenderness (visceral injury, duodenal haematoma).
- **Anogenital examination**: only after consent/assent, ideally **once**, by the most experienced person available, in the **frog-leg** or knee-chest position; look for bruising, lacerations, bleeding, discharge, warts, hymenal transections (a normal hymen does **not** exclude abuse: most sexually abused children have normal genital findings).

- **Development and behaviour** (frozen watchfulness in toddlers).

Red flags

RED FLAG

- **Any bruise in a baby who is not yet mobile** (under about 6 months, not cruising).
- **TEN-4-FACESp** bruising (see High-yield).
- **Patterned injuries**: hand marks, loop marks (electric cable, belt), bite marks, ligature marks on wrists/ankles or neck.
- **Burns**: immersion scalds with clear "glove and stocking" margins or sparing of the buttocks/flexures; cigarette burns (round, 5–10 mm, uniform depth); contact burns shaped like an iron or hot plate.
- **Fractures in non-mobile infants**, posterior rib fractures, metaphyseal fractures, multiple fractures of different ages.
- **Infant with unexplained encephalopathy**, seizures or apnoea (abusive head trauma).
- **STI in a prepubertal child** (gonorrhoea, syphilis beyond the neonatal period, trichomonas, HIV not explained by vertical transmission), **pregnancy under 14**.
- **A child who says they were abused.** Believe them and act.

Thinking like a clinician

- **The history is the main diagnostic tool**: an injury that is **unexplained, inconsistent with development, or explained differently each time** is abuse until proven otherwise.
- **Consider mimics, but don't let them stop you**: dermal melanocytosis (Mongolian blue spots: present since birth, slate-grey, don't change), bleeding disorders (check FBC and clotting), osteogenesis imperfecta (blue sclerae, family history), rickets, impetigo (vs cigarette burns), cultural practices (scarification, cupping, herbal burns), accidental straddle injuries (anterior, unilateral), lichen sclerosus and urethral prolapse (vs sexual abuse).
- **Neglect vs poverty**: a family that is poor but uses available resources (brings the child for care, accepts food support) is struggling, not neglectful. A family that repeatedly fails to give prescribed ART, misses every review and leaves a toddler alone all day is neglecting the child.
- **Adolescent "relationships"**: a 15-year-old with a 25-year-old "boyfriend" is a defilement case and possibly exploitation (transactional sex, "sugar daddy"). A consensual relationship between two 16-year-olds is still legally defilement, but the Children Act diverts child offenders; handle with judgement and counselling, involving the PSWO.
- **Don't be the investigator.** Your job is medical care, careful documentation and referral; the police and PSWO investigate.

Clinical workup

Step 0: First 5 minutes

1. **ABC and life-threatening injuries first** (head injury, abdominal bleeding, burns with shock, poisoning). Use ETAT.
2. **Safety**: is the child safe right now? Who is with them? If the suspected abuser is present and the child is frightened, separate gently (e.g. for an "X-ray" or "weighing").
3. **Time since sexual assault**: if within **72 hours**, prioritise **PEP** and **emergency contraception** (start within the hour if possible) and **forensic samples** before washing.
4. Call for help: the senior on call, the medical social worker; don't manage suspected abuse alone.

Step 1: Focused history

- [ ] Mechanism, timing, witnesses, delay. *Plausibility.*
- [ ] Developmental stage. *Could the child have done this?*
- [ ] Previous injuries/admissions. *Pattern.*
- [ ] Household members and carers. *Risk and protection.*
- [ ] Child's own account (verbatim). *Disclosure and evidence.*
- [ ] For sexual assault: time, type of contact, menarche, symptoms. *PEP, EC, samples.*
- [ ] HIV status, immunisation (HepB, tetanus). *Prophylaxis.*

Step 2: Focused examination

- General: growth (Z-scores), hygiene, demeanour.
- Skin and body map; head, eyes (fundi in infants), mouth, ears; skeleton; abdomen.
- Anogenital (once, by the most experienced examiner, with consent/assent; colposcope or good light).
- Document **positive and negative** findings.

Step 3: Bedside tests

Test	Use
Urine pregnancy test	Every post-menarchal girl before EC/PEP and as evidence
HIV rapid test	Baseline before PEP (do not delay PEP for the result)
RBS, Hb	Neglect, malnutrition, bleeding
Urine dipstick	Haematuria (abdominal/renal trauma)
Photography (with consent, scale ruler in the picture)	Legal documentation

Step 4: Laboratory & imaging

Test	When	Expected / abnormal finding	How it changes management	Availability
FBC, platelets, PT/INR, APTT	Unexplained bruising or bleeding	Normal results support inflicted injury; abnormal suggests a bleeding disorder	Excludes mimic (ITP, haemophilia, vitamin K deficiency, leukaemia)	HC IV (FBC); clotting at hospitals
Skeletal survey	All children **under 2 years** with suspected physical abuse; consider to 3 years	Occult fractures (ribs, metaphyses), fractures of different ages	Strong evidence of abuse	Regional/national referral hospitals
Repeat skeletal survey (11–14 days)	If first survey is equivocal	Healing fractures become visible	Confirms injury	As above
CT head	Infant with encephalopathy, seizures, scalp swelling, or under 1 year with any suspected abuse	Subdural haemorrhage (different ages), skull fractures, hypoxic injury	Abusive head trauma; neurosurgery	Mulago, some regional referral hospitals, private

Test	When	Expected / abnormal finding	How it changes management	Availability
Fundoscopy (by ophthalmologist)	Suspected abusive head trauma	Extensive multilayered retinal haemorrhages	Supports AHT	Regional/national referral hospitals
Liver enzymes, amylase, urinalysis	Abdominal injury	Raised (occult visceral injury)	Imaging (ultrasound/CT)	Hospitals
STI tests: HIV, syphilis (RPR/TPHA), HBsAg; vaginal/urethral swabs for gonorrhoea, chlamydia, trichomonas (NAAT/culture where available)	Sexual abuse	Positive	Treatment; legal significance in prepubertal children	HIV, RPR, HBsAg at HC III–IV; swabs at hospitals
Forensic swabs (vaginal, anal, oral, skin; clothing in paper bags)	Sexual assault within 72 h (up to 7 days for some samples)	Semen/DNA	Prosecution	Collected at facility; analysed at the Government Analytical Laboratory (Wandegeya) via police
Age assessment (dental/X-ray)	Age disputed (legal relevance: under 14 = aggravated)	Estimated age range	Charge category	Hospitals, police surgeon

Step 5: Putting it together

Model one-liner: "A 4-month-old boy (5.2 kg, WAZ –1.4), non-mobile, brought with a 'swollen thigh after falling from the bed 3 days ago', has a spiral femoral fracture, two bruises on the cheek and a torn upper frenulum; the history does not explain the findings and care was delayed."

Problem list: (1) femoral fracture; (2) suspected physical abuse (bruising in a non-mobile infant, delay, implausible mechanism); (3) safety of the infant and siblings; (4) rule out a bleeding disorder or bone disease.

Worked interpretation: "fell from the bed" (a short fall of under 1 m) rarely causes serious injury in an infant and cannot explain facial bruises plus a frenulum tear; a non-mobile infant cannot generate the twisting force of a spiral femoral fracture. Plan: admit (place of safety), analgesia and orthopaedic care, skeletal survey, CT head (age <1 year), fundoscopy, FBC and clotting, body map and photographs, inform the PSWO and police (CFPU), and screen siblings.

Differential diagnosis

Finding	Abuse	Mimic / accidental explanation	How to tell apart
Blue-grey patches	Bruises (change colour over days, tender, swollen)	**Dermal melanocytosis** (Mongolian blue spots): present since birth, sacrum/buttocks/back, non-tender, unchanged	Document at birth; review in a few days
Multiple bruises	Inflicted injury	**ITP, haemophilia, vitamin K deficiency, leukaemia, von Willebrand**	FBC, film, clotting, family history
Fractures	Inflicted injury	**Osteogenesis imperfecta, rickets, osteopenia of prematurity**, birth injury (clavicle), accidental fall (toddler's spiral tibial fracture)	Blue sclerae, dentinogenesis, family history; rickets signs and alkaline phosphatase; developmental stage

Finding	Abuse	Mimic / accidental explanation	How to tell apart
Round burns	Cigarette burns (uniform, deep, 5–10 mm)	**Impetigo** (crusted, varying size, spreads), insect bites	Culture, response to antibiotics, pattern
Scald	Immersion (sharp margins, symmetrical, buttock sparing)	Spill/splash (irregular, arrow-shaped, asymmetrical, on the front)	Pattern and history
Linear marks	Beating with a stick or cable	**Traditional scarification**, cupping, herbal burns	History, cultural context (still harmful if painful/dangerous)
Genital bleeding or bruising	Sexual abuse	**Accidental straddle injury** (anterior, unilateral labial), **urethral prolapse**, **lichen sclerosus**, vulvovaginitis, foreign body, precocious puberty	Examination pattern, history consistency
Encephalopathy in an infant	Abusive head trauma	Meningitis, cerebral malaria, metabolic disease, accidental major trauma (road traffic)	CT, LP, fundoscopy, history
Failure to thrive	Neglect	Organic disease: HIV, TB, cardiac, renal, malabsorption, SAM from food insecurity	Growth on admission (gains in hospital = neglect/feeding), tests

Investigations

Test	What you are looking for	Why
FBC, clotting	Bleeding disorder	Excludes mimic
Skeletal survey (<2 years)	Occult fractures	Evidence of repeated abuse
CT head, fundoscopy	Subdural/retinal haemorrhages	Abusive head trauma
HIV, syphilis, HBsAg, pregnancy, STI swabs	Infection, pregnancy	Prophylaxis and treatment; evidence
Forensic samples	DNA, semen	Prosecution
Growth chart	Weight gain in hospital	Neglect vs organic disease

Management

A. Immediate medical care

1. Treat life-threatening injuries (ETAT, surgical care, burns care).
2. **Analgesia**: paracetamol **15 mg/kg** 6-hourly (max 60 mg/kg/day); ibuprofen **10 mg/kg** 8-hourly (max 40 mg/kg/day; avoid if bleeding risk or dehydration); morphine for severe pain (0.1 mg/kg IV/SC, titrated).
3. Admit if the child needs treatment **or is unsafe at home**.

B. Sexual abuse/defilement: medical management (step by step)

1. **Consent/assent**: explain each step; the child can say "stop". Consent for examination and forensic sampling from the parent/guardian (if not the suspect) or, if needed, proceed in the child's best interests with the PSWO/police.
2. **Forensic evidence first** (if within 72 hours, before washing, ideally before medication that requires undressing): swabs, clothing in **paper** bags, chain-of-custody form, labelled and sealed, handed to the police.

3. **HIV PEP** if within **72 hours** (sooner is better; ideally within 2 hours).
4. **Emergency contraception** for post-menarchal girls (or Tanner stage 3+), within 72 hours, up to 120 hours.
5. **STI prophylaxis.**
6. **Hepatitis B vaccination** (if not fully immunised) and **tetanus** if wounds.
7. **Baseline tests**: HIV, syphilis, HBsAg, pregnancy.
8. **Psychosocial first aid** and referral for counselling.
9. **Report** (police CFPU, PSWO) and complete **Police Form 3A**.
10. **Follow-up**: PEP adherence and side-effects (2 weeks), HIV test at **6 weeks and 3 months** (and 6 months per some guidance), pregnancy test at 2–3 weeks if EC failed, syphilis repeat, psychological support, school, legal process.

Drug table for sexual assault in children and adolescents

Purpose	Drug	Dose	Route	Frequency	Duration	Notes / max
HIV PEP, children <30 kg	ABC/3TC + DTG (preferred where available) or **ABC + 3TC + LPV/r** (UCG 2023)	Weight-band doses as for ART (see *HIV/AIDS in Children*): e.g. 14–19.9 kg ABC/3TC 120/60 2.5 tabs + pDTG 10 mg 2.5 tabs	Oral	Once daily (LPV/r twice daily)	**28 days**	Start within **72 h**; do not wait for the baseline HIV result
HIV PEP, adolescents ≥30 kg	**TDF/3TC/DTG (TLD)** (current preferred; UCG 2023 adult option TDF + 3TC + ATV/r)	1 tablet	Oral	Once daily	**28 days**	
Emergency contraception	**Levonorgestrel**	**1.5 mg** single dose (**3 mg** if on EFV, rifampicin or other enzyme inducers; UCG says double the dose on ARVs)	Oral	Once	–	Within **72 h**, some efficacy to **120 h**; copper IUD up to 5 days is an alternative for older adolescents
Gonorrhoea prophylaxis, ≥45 kg	Ceftriaxone (UCG 2023)	**1 g** (or cefixime 400 mg orally)	IM	Single dose	–	Children <45 kg: ceftriaxone **25–50 mg/kg** IM single dose (max 250 mg) (VERIFY paediatric dose)
Chlamydia prophylaxis	Azithromycin	**20 mg/kg** (max **1 g**)	Oral	Single dose	–	Adolescents: azithromycin 1 g or doxycycline 100 mg BD 7 days (UCG; doxycycline only ≥8 years)
Trichomonas prophylaxis	Metronidazole	Adolescents **2 g** single dose (UCG); children **15 mg/kg/day** in 3 doses for 7 days (VERIFY)	Oral	–	–	Avoid alcohol
Hepatitis B	Hepatitis B vaccine	Age-appropriate dose (0.5 mL <20 years)	IM	0, 1, 6 months	–	Plus HBIG if assailant known HBsAg-positive and available
Tetanus	Td ± tetanus immunoglobulin	Td 0.5 mL; TIG 75 IU (<5 y), 125 IU (5–10 y), 250 IU (>10 y)	IM	Per UCG	–	For tetanus-prone wounds

DRUG DOSING

UCG 2023 still lists **LPV/r-based** paediatric PEP and **ATV/r-based** adult PEP. WHO's 2024 PEP guidance and the Uganda 2022 consolidated guidelines prefer **DTG-based** regimens (TLD for adolescents ≥30 kg; ABC/3TC + DTG for younger children), which are better tolerated and improve completion. Use whatever your ART clinic stocks; **starting promptly matters more than the exact regimen**.

C. Physical abuse and neglect

1. Treat injuries and pain; admit the child (and consider siblings' safety).
2. Investigations above (skeletal survey, CT head, clotting).
3. **Document**: history verbatim, body map, measured injuries, photos.
4. **Inform** the PSWO and police CFPU the same day; involve the medical social worker.
5. **Multidisciplinary meeting** (paediatrician, social worker, PSWO, police) to plan safety and discharge: back home with support, to a relative (kinship care), or to a place of safety by **care order** (Family and Children Court).
6. For neglect: nutrition rehabilitation, catch-up immunisation, treat illness, link the family to social protection (e.g. cash transfers, OVC programmes), parenting support, and follow-up home visits.

D. Documentation and the Police Form

- **Police Form 3 (PF3)**: medical examination of a person with injuries (assault). **Police Form 3A (PF3A)**: medical examination of victims of **sexual offences**. The police issue the form; the examining medical officer completes it and may be summoned to court.
- Write **legibly**, factually and in plain language: describe injuries (site, size in cm, shape, colour, pattern), state whether findings are **consistent** with the history, record what the child said **in their own words**, and date, time and sign.
- **Never** write "no evidence of rape" when the examination is normal: write "normal examination; a normal examination does not exclude sexual abuse".
- **Do not delay treatment** waiting for the police form; treat first, then complete the form when it arrives.

E. Reporting pathway (Uganda)

```
Child presents  →  Treat, make safe (admit if needed), document
       │
       ├─► Medical social worker / senior clinician (same day)
       ├─► Police Child & Family Protection Unit  →  PF3 / PF3A, investigation, court
       ├─► District Probation & Social Welfare Officer  →  safety plan, care order, follow-up
       ├─► LC1 chairperson (community)   ·   Sauti 116 helpline (toll-free, 24 h)
       └─► Follow-up: medical (PEP, HIV tests), psychosocial, school, legal
```

UGANDA CONTEXT

"Settling" defilement out of court (the family accepts money or a marriage arrangement from the offender) is illegal and leaves the child unprotected. Explain this kindly to families, and report anyway. Health workers have a duty to act in the child's best interests; withholding information that a child is being abused is not protected by confidentiality (see *Professionalism & Ethics*).

F. Prevention (what clinicians can do)

- Ask about **discipline and violence** at routine visits; teach **positive parenting** (e.g. Parenting for Lifelong Health, Raising Voices' Good School toolkit).
- Recognise and support **at-risk families** (young mothers, disability, alcohol, domestic violence).

- **Comprehensive sexuality education**, adolescent-friendly services, keeping girls in school.
- **Birth registration** (proof of age protects against child marriage and child labour).
- Advocate against corporal punishment and harmful practices (FGM, ebiino).

Complications

- **Death** (abusive head trauma, abdominal injury, burns, starvation).
- **Disability**: cerebral palsy, blindness, epilepsy after head trauma; contractures after burns.
- **Sexual and reproductive**: HIV and other STIs, pregnancy (obstructed labour, fistula, unsafe abortion), genital injury.
- **Psychological**: PTSD, depression, anxiety, self-harm and suicide, substance misuse, conduct disorder, poor school performance.
- **Social**: school drop-out, early marriage, homelessness, revictimisation and intergenerational violence.
- **Iatrogenic**: secondary trauma from repeated interviews and examinations, stigma when confidentiality is breached.

Ward-round & exam pearls

Model case presentation (child):

> "Atim is a 7-year-old girl from Wakiso, weight 19 kg (WAZ −0.9), height 117 cm (HAZ −0.8), fully immunised for age per her card, HIV status negative (tested today), brought by her mother with vaginal bleeding and pain on passing urine since last evening. She told me, in her words, 'the neighbour's son took me inside and hurt me down there' yesterday at about 4 pm, about 20 hours ago. On examination she is distressed but stable; there is a fresh posterior fourchette laceration and bruising of the labia minora. Summary: sexual assault of a 7-year-old within 72 hours, with genital injury, which constitutes aggravated defilement (victim under 14). Plan: forensic swabs collected and handed to police with chain of custody, HIV PEP (ABC/3TC + pDTG for 28 days by weight band), STI prophylaxis, hepatitis B vaccine catch-up, analgesia, PF3A completed, PSWO and police CFPU informed, psychosocial support, and follow-up for HIV testing at 6 weeks and 3 months."

Likely viva questions (with answers)

1. *Who is a child in Uganda?* Anyone under 18.
2. *What are the four general principles of the UNCRC?* Non-discrimination, best interests, survival and development, participation.
3. *Define aggravated defilement.* Defilement where the victim is under 14, has a disability, or the offender is HIV-positive, a parent/guardian/person in authority, or a serial offender.
4. *What is TEN-4-FACESp?* A bruising clinical decision rule: Torso, Ears, Neck bruising under 4 years, any bruise under 4 months, or bruising of the Frenulum, Angle of jaw, Cheek, Eyelid, Subconjunctiva, or Patterned bruising.
5. *What do you give a 15-year-old girl seen 30 hours after rape?* PEP (TLD 28 days if ≥30 kg), levonorgestrel 1.5 mg, STI prophylaxis, HepB vaccine, pregnancy and HIV tests, counselling, PF3A, report.
6. *Which fractures are most specific for abuse?* Posterior rib fractures, metaphyseal corner fractures, scapular, spinous process and sternal fractures, multiple fractures of different ages.
7. *What is the role of the PSWO?* Statutory child-protection officer at district level: investigates welfare, arranges safe placement and care orders, coordinates services.

OSCE tips: stations include **taking a history from a parent where the story changes**, **counselling a mother after her child's defilement**, and **completing a body map/PF3**. Stay non-judgemental, keep the child's safety central, explain what you must report and why, and never promise confidentiality you cannot keep.

Mnemonics & memory aids

- **UNCRC principles "N-B-S-P"**: **N**on-discrimination (Art. 2), **B**est interests (Art. 3), **S**urvival and development (Art. 6), **P**articipation (Art. 12). Articles **2-3-6-12**.
- **Rights categories "SDPP"**: **S**urvival, **D**evelopment, **P**rotection, **P**articipation.
- **TEN-4-FACESp**: **T**orso, **E**ars, **N**eck (under **4** years); any bruise under **4** months; **F**renulum, **A**ngle of jaw, **C**heek, **E**yelid, **S**ubconjunctival; **p**atterned.
- **Aggravated defilement "14-HIV-PAD"**: under **14**, offender **HIV**+, **P**arent/guardian/person in authority, **A**gain (serial), victim with **D**isability.
- **After sexual assault "PEP-PEP-SHOT"**: **P**EP, **E**mergency contraception, **P**regnancy test; **P**rophylaxis for STIs, **E**vidence (forensic), **P**olice form 3A; **S**upport (psychosocial), **H**epatitis B vaccine, **O**ther injuries, **T**etanus.
- **Neglect "FAIL"**: **F**ailure to thrive, **A**bandonment/absent supervision, **I**mmunisations missed, **L**ack of medical care.

High-yield summary

Numbers & doses to know

Item	Value
Definition of a child	Under **18** years
Age of sexual consent (Uganda)	**18** years
Aggravated defilement	Victim **<14**, disability; offender HIV+, parent/guardian/authority, serial
Maximum penalty for aggravated defilement	**Death**
Age of criminal responsibility	**12** years
Legal marriage age	**18** (Constitution Art. 31)
UNCRC adopted / Uganda ratified	**1989 / 1990**
African Charter (ACRWC) ratified	**1994**
PEP window	Within **72 hours**; course **28 days**
Emergency contraception (levonorgestrel)	**1.5 mg** single dose within **72 h** (up to 120 h); **3 mg** with enzyme inducers
HIV tests after assault	Baseline, **6 weeks**, **3 months**
Forensic samples	Ideally within **72 hours**
Skeletal survey	All children **<2 years** with suspected physical abuse
Bruising in a non-mobile infant	Abuse until proven otherwise
Helpline	**Sauti 116** (toll-free)
Police forms	**PF3** (injuries), **PF3A** (sexual offences)
Uganda VACS 2015	About **1 in 3 girls, 1 in 6 boys** sexual violence in childhood
WHO: physical/psychological punishment	**6 in 10** under-5s

Classic exam traps

- **"She consented, so it isn't defilement."** Wrong: under 18 consent is legally irrelevant.
- **"A normal hymen excludes sexual abuse."** Wrong: most abused children have **normal** genital findings.
- **"Wait for the HIV test result before starting PEP."** Wrong: start PEP at once; adjust if the baseline test is positive (then it becomes ART).
- **"Ask the child leading questions to get the full story."** Wrong: use open questions; record verbatim; avoid repeated interviews.
- **"The family has settled the matter with the neighbour; no need to report."** Wrong: defilement is a criminal offence; report and protect the child.
- **"Blue spots on the buttocks of a baby = abuse."** Could be **dermal melanocytosis**; check the history and whether they change.
- **"Confront the father on the ward."** Wrong: keep the child safe, involve the PSWO/police and the senior team.
- **"Fill the PF3 before treating."** Wrong: treat first; documentation follows.

Questions seniors ask

What makes you suspect that an injury was not accidental? No or changing explanation, a mechanism incompatible with the child's development, delay in seeking care, injuries of different ages, patterned or high-risk sites (TEN-4-FACESp), and the child's or a witness's disclosure.

Which investigations do you request for a 6-month-old with an unexplained fracture? Skeletal survey (repeat in 2 weeks), CT head, fundoscopy, FBC and clotting, bone profile (calcium, phosphate, ALP, vitamin D), and screen siblings.

A 13-year-old is pregnant and says the father is her 19-year-old boyfriend. What do you do? This is aggravated defilement (under 14). Provide antenatal care and counselling, assess safety, test for HIV/syphilis, report to the police CFPU and PSWO, and involve the social worker; her pregnancy is high risk (anaemia, pre-eclampsia, obstructed labour).

What PEP would you give a 22 kg child? ABC/3TC 120/60 mg 3 tablets + pDTG 10 mg 3 tablets (or DTG 50 mg one tablet) once daily for 28 days (20–24.9 kg band), or ABC + 3TC + LPV/r per UCG 2023 if DTG is unavailable.

Who do you report to, and does confidentiality stop you? Police CFPU and the district PSWO (and the hospital social worker); Sauti 116 for advice. Confidentiality does not cover concealing abuse of a child; share what is needed for protection, with the people who need to know.

How do you tell neglect from poverty? Ask whether the carer uses the resources available; poverty with good care-seeking and adherence is not neglect. Support poor families with social protection; act on persistent failure to meet the child's needs despite support.

What is the role of the doctor in court? To present the medical findings factually: what was found, whether it is consistent with the account, and the limits of what medicine can say. You are a professional witness, not an advocate for either side.

SELF-TEST

Flashcards

27 cards. Cover the right-hand column and test yourself.

#	Question	Answer
1	Who is a child under Ugandan and international law?	Any person **under 18 years** (UNCRC Art. 1, African Charter, Uganda Constitution, Children Act).
2	The four general principles of the UNCRC	**Non-discrimination** (Art. 2), **best interests** (Art. 3), **life, survival and development** (Art. 6), **respect for the child's views/participation** (Art. 12).
3	The four categories of children's rights	**Survival**, **development**, **protection**, **participation**.
4	When did Uganda ratify the UNCRC and the African Charter (ACRWC)?	UNCRC **1990** (adopted 1989); ACRWC **1994**.
5	Define defilement (Penal Code Act s.129)	An **unlawful sexual act with a person under 18**. Consent is legally irrelevant.
6	When is defilement 'aggravated'?	Victim **under 14**; victim has a **disability**; offender is **HIV-positive**, a **parent/guardian/person in authority**, or a **serial offender**. Maximum penalty: **death**.
7	WHO forms of child maltreatment	**Physical**, **sexual**, **emotional** abuse, **neglect**, and **exploitation** (commercial/labour).
8	TEN-4-FACESp bruising rule	Bruising of the **Torso, Ears, Neck** in a child **under 4**; **any** bruise under **4 months**; bruising of **Frenulum, Angle of jaw, Cheek, Eyelid, Subconjunctiva**; **patterned** bruising.
9	Fractures most specific for abuse	**Posterior rib** fractures, **metaphyseal corner** fractures, scapular, sternal and spinous process fractures, **multiple fractures of different ages**, fractures in **non-mobile** infants.
10	Triad of abusive head trauma	**Subdural haemorrhage**, **retinal haemorrhages**, **encephalopathy**, with an inadequate history.
11	Burn patterns suggestive of abuse	**Immersion scalds** with sharp 'glove and stocking' margins, symmetrical, sparing flexures/buttocks; **cigarette burns** (round, 5–10 mm, uniform depth); **contact burns** shaped like the object.
12	Which investigation is mandatory under 2 years with suspected physical abuse?	A **skeletal survey** (repeat in 11–14 days if equivocal), plus CT head under 1 year, fundoscopy and clotting screen.
13	Does a normal genital examination exclude sexual abuse?	**No.** Most sexually abused children have **normal** genital findings. Write: 'normal examination; does not exclude sexual abuse'.
14	STIs in a prepubertal child that strongly suggest sexual abuse	**Gonorrhoea**, **syphilis** (beyond the neonatal period), **trichomonas**, **chlamydia** (after infancy), **HIV** not explained by vertical transmission.
15	HIV PEP after sexual assault: window and duration	Start **within 72 hours** (as soon as possible), continue for **28 days**; do not wait for the baseline HIV result.
16	PEP regimen for an adolescent ≥30 kg	**TDF/3TC/DTG (TLD)** once daily for 28 days (UCG 2023 still lists TDF + 3TC + ATV/r).
17	Emergency contraception after rape	**Levonorgestrel 1.5 mg** single dose within **72 h** (some effect to 120 h); **3 mg** if on enzyme inducers (EFV, rifampicin). Copper IUD up to 5 days as alternative.
18	Police forms for medical examination in Uganda	**PF3**: examination of an injured person (assault). **PF3A**: examination of a victim of a **sexual offence**.

#	Question	Answer
19	Who is the statutory child-protection officer at district level?	The **Probation and Social Welfare Officer (PSWO)**.
20	Uganda's national child helpline	**Sauti 116** (toll-free, 24 hours).
21	How should you interview a child who discloses abuse?	Open questions, no leading questions, **record their exact words**, do not promise secrecy, tell them they did right to tell, avoid repeated interviews.
22	Mimics of physical abuse	**Dermal melanocytosis** (Mongolian spots), **bleeding disorders** (ITP, haemophilia, VKDB, leukaemia), **osteogenesis imperfecta**, **rickets**, **impetigo**, cultural practices (scarification, cupping).
23	Age of criminal responsibility and legal marriage age in Uganda	Criminal responsibility **12 years**; marriage **18 years** (Constitution Art. 31).
24	Uganda VACS 2015 headline on sexual violence	About **1 in 3 girls** and **1 in 6 boys** experienced sexual violence in childhood (verify exact percentages).
25	Neglect vs poverty	Poverty with good care-seeking and use of available resources is **not** neglect. Neglect = persistent failure to meet needs despite available means or support.
26	What is 'ebiino'?	A harmful traditional practice in Uganda: **extraction of infants' canine tooth buds** ('false teeth'), causing bleeding, infection, sepsis and death.
27	HIV testing schedule after sexual assault	**Baseline**, then **6 weeks** and **3 months** after exposure.

SELF-TEST

Single best answer questions

15 questions. The answers and explanations follow at the end.

1. A 3-month-old boy (5.4 kg) is brought to Mengo casualty because he 'won't move his left leg'. The father says he 'rolled off the bed yesterday'. The baby cannot yet roll over. There is a swollen left thigh, two 1 cm bruises on the right cheek and a torn upper lip frenulum. Which is the MOST appropriate next step?

A Apply a splint, give paracetamol and discharge with an orthopaedic review in a week

B Confront the father and ask him to admit what happened

C Admit, give analgesia, request femur X-ray, skeletal survey, CT head, clotting screen, and inform the social worker, PSWO and police

D Request a femur X-ray only; abuse is unlikely if the parents brought him in

E Report to the police and allow the parents to take him home while they investigate

2. A 16-year-old girl (52 kg) presents to a HC IV 30 hours after being raped by an unknown man. She is HIV-negative on a rapid test and her urine pregnancy test is negative. She is distressed but has no serious injuries. Which management is MOST complete?

A Counselling only; PEP is ineffective after 24 hours

B PEP only, because she is young and STIs are unlikely

C Refer her to the police first; medical treatment can wait until the PF3A is issued

D Emergency contraception only, because PEP needs a positive HIV test first

E TLD for 28 days, levonorgestrel 1.5 mg, ceftriaxone, azithromycin and metronidazole, hepatitis B vaccine, forensic samples, PF3A, psychosocial support and follow-up HIV tests

3. A 14-year-old girl attends ANC at 20 weeks. She says the father is her 'boyfriend', a 24-year-old boda-boda rider, and she agreed to the relationship. What is the legal position and the MOST appropriate action?

A No offence, because she consented and is over 13

B Defilement, because she is under 18; provide ANC and counselling, assess safety, and report to the police CFPU and PSWO

C Aggravated defilement, because the man is over 18; report only if she agrees

D No offence if the families agree to a marriage

E Defilement only if she was under 12 at the time of conception

4. A 6-month-old girl (6.9 kg) has several blue-grey patches over the sacrum and buttocks, each 3–6 cm, non-tender, with no swelling. Her mother says they have been there 'since birth'. She is thriving and developing normally. What is the MOST likely explanation?

A Dermal melanocytosis (Mongolian blue spots)

B Inflicted bruises from beating

C Immune thrombocytopenia

D Haemophilia A

E Vitamin K deficiency bleeding

5. A 5-month-old boy (6.0 kg) is brought with seizures and vomiting. His mother says he 'was fine, then suddenly stopped breathing'. He is drowsy, the fontanelle is bulging, and there are no external injuries. Malaria RDT is negative; RBS 5.4 mmol/L. CSF is bloody in all three tubes. Which investigation will MOST help confirm the likely diagnosis? Advanced

A Repeat lumbar puncture in 24 hours
B Blood culture and start ceftriaxone only
C EEG
D CT head and fundoscopy, with skeletal survey
E Serum calcium and magnesium

6. A 4-year-old girl (15 kg) is brought with a 3-week vaginal discharge. Swabs grow Neisseria gonorrhoeae. Her mother is HIV-negative and has no STI. What is the MOST appropriate interpretation and action?

A Non-specific vulvovaginitis due to poor hygiene; advise better hygiene
B Acquired from shared towels; treat and reassure
C Perinatally acquired infection; treat only
D A urinary tract infection misreported as vaginal discharge
E Strong evidence of sexual abuse; treat, test for other STIs and HIV, assess safety, and report to the police and PSWO

7. An 8-year-old boy tells you on the ward, 'If I tell you something, will you promise not to tell anyone?' What is the BEST response?

A 'Yes, I promise. Doctors never tell anyone anything.'
B 'Then don't tell me; I don't want to get involved.'
C 'I can't promise that, because if someone is hurting you I need to get help to keep you safe; but I will only tell people who can help.'
D 'You must tell your parents first.'
E 'Only if your mother is here with us.'

8. A 2-year-old boy (8.6 kg, WAZ −3.2) is admitted with pneumonia. He has missed all vaccines after 6 weeks, has untreated scabies, and is often left alone at home while both parents drink at a local bar. During admission he gains 900 g in 10 days on normal ward food. Which conclusion is MOST appropriate?

A Organic failure to thrive; investigate for malabsorption
B Neglect; involve the social worker and PSWO, support the family and plan follow-up
C Poverty; no protection concern
D Physical abuse; report for prosecution immediately
E Normal growth for an African child

9. A 9-year-old girl was defiled by a neighbour 3 hours ago. She weighs 26 kg. Her baseline HIV test is negative. Which PEP regimen is MOST appropriate if the ART clinic has all paediatric formulations? Advanced

A TDF/3TC/DTG one tablet daily for 28 days
B Nevirapine syrup for 6 weeks
C Co-trimoxazole prophylaxis
D ABC/3TC 600/300 mg one tablet + DTG 50 mg one tablet once daily for 28 days
E AZT/3TC twice daily for 7 days

10. Which statement about examining a child after suspected sexual abuse is correct?

A The anogenital examination should be done once, by the most experienced examiner available, with consent/assent

B Every clinician who sees the child should examine the genitalia to confirm findings

C A speculum examination is routine in prepubertal girls

D A normal hymen excludes penetration

E The examination should be done before treating any injuries

11. A 3-year-old boy (13 kg) is brought with burns to both feet and lower legs. The burns have a sharp, horizontal upper margin at the same level on both legs, with no splash marks; the soles are spared where they touched the bottom of the basin. The mother says he 'stepped into hot water'. What is the MOST likely mechanism?

A Accidental spill of a hot kettle

B Forced immersion in hot water

C Contact with a hot iron

D Chemical burn from herbal medicine

E Accidental fall into a cooking fire

12. Which of the following is one of the four general principles of the UN Convention on the Rights of the Child?

A Free primary education for all

B Protection from child labour

C Birth registration within 6 months

D The child's duty to respect parents

E Respect for the views of the child (Article 12)

13. After you report a case of defilement of a 10-year-old, the girl's uncle tells you that the families have 'settled the matter': the neighbour paid two goats, and they now want to withdraw the case and take her home today. She is medically stable on PEP. What should you do?

A Accept the settlement; it is a family decision

B Discharge her and stop PEP because the case is closed

C Tell the uncle the police will arrest him

D Explain that defilement is a criminal offence that cannot be settled privately, continue PEP, and inform the PSWO and police CFPU so that her safety at home is assessed before discharge

E Destroy the PF3A to respect the family's wishes

14. A 12-year-old boy (35 kg) attends with a fever. You notice multiple linear bruises of different colours across his back and buttocks. He says his teacher 'canes us when we fail'. What is the MOST appropriate response?

A Corporal punishment is part of Ugandan culture; treat the fever only

B Advise the boy to work harder at school

C Treat the fever, document the injuries (body map), and report to the PSWO/police; corporal punishment in schools is prohibited and causes harm

D Write to the headteacher yourself and demand the teacher's dismissal

E Give the boy analgesia and tell his parents to transfer him to another school

15. A 7-year-old girl (21 kg) was defiled 96 hours ago and is brought to Mengo today. She is HIV-negative. Which statement about her management is MOST accurate?

A PEP is no longer recommended (beyond 72 hours), but she still needs STI prophylaxis, hepatitis B vaccination, forensic examination, documentation, psychosocial support, reporting and HIV testing at 6 weeks and 3 months

B PEP should be started because it works up to 7 days

C Emergency contraception is the most urgent intervention

D No medical treatment is needed because the event was more than 72 hours ago

E She should be tested for HIV in 2 years

Answers & explanations

1. C · Admit, give analgesia, request femur X-ray, skeletal survey, CT head, clotting screen, and inform the social worker, PSWO and police

C is correct: a non-mobile infant with a femoral injury, facial bruising (TEN-4-FACESp) and a torn frenulum, with an implausible mechanism, needs admission as a place of safety, analgesia and a full abuse workup (skeletal survey, CT head under 1 year, clotting), and same-day referral to the social worker, PSWO and police. A is wrong: discharging an infant with suspected abuse back to the possible abuser risks further harm or death. B is wrong: confrontation can endanger the child and is not the clinician's role. D is wrong: bringing a child for care does not exclude abuse, and occult fractures and head injury need to be looked for. E is wrong: the child must be kept safe during the investigation.

2. E · TLD for 28 days, levonorgestrel 1.5 mg, ceftriaxone, azithromycin and metronidazole, hepatitis B vaccine, forensic samples, PF3A, psychosocial support and follow-up HIV tests

E is correct: within 72 hours she needs PEP (TLD 28 days for ≥30 kg), emergency contraception, STI prophylaxis, HBV vaccination, evidence collection, documentation, support and follow-up testing at 6 weeks and 3 months. A is wrong: PEP is recommended up to 72 hours. B is wrong: STIs and pregnancy are real risks after rape. C is wrong: treatment must never wait for the police form; time-critical drugs (PEP, EC) come first. D is wrong: PEP is given to HIV-negative people; it does not require a positive test.

3. B · Defilement, because she is under 18; provide ANC and counselling, assess safety, and report to the police CFPU and PSWO

B is correct: any sexual act with a person under 18 is defilement whatever the 'consent'; she needs high-risk ANC, HIV/syphilis testing, counselling, safety assessment and a report (if she was under 14 at conception, it is aggravated). A is wrong: consent is irrelevant under 18. C is wrong: aggravation depends on her age (<14), disability, or the offender's HIV status/authority/repeat offending, not his adulthood alone; and the duty to protect is not waived by her wishes. D is wrong: marriage under 18 is unconstitutional and 'settling' does not remove the offence. E is wrong: the threshold is 18 (and 14 for aggravated), not 12.

4. A · Dermal melanocytosis (Mongolian blue spots)

A is correct: congenital, non-tender, slate-grey patches over the sacrum/buttocks that do not change are dermal melanocytosis, very common in African infants; documenting them early prevents false accusations. B is wrong: bruises are tender, may be swollen and change colour over days. C is wrong: ITP causes petechiae and bruising in varied sites with bleeding, not stable sacral patches since birth. D is wrong: haemophilia causes bruising with mobility and joint/muscle bleeds, not congenital sacral patches. E is wrong: VKDB presents with bleeding (umbilical, GI, intracranial) in the first months, not unchanged patches since birth.

5. D · CT head and fundoscopy, with skeletal survey

D is correct: an infant with encephalopathy, seizures, a bulging fontanelle and bloody CSF without a history of trauma suggests abusive head trauma; CT (subdural haemorrhages) and fundoscopy (retinal haemorrhages) confirm it, and a skeletal survey looks for associated fractures. A is wrong: a repeat LP adds risk and does not identify subdural bleeding or its cause. B is wrong: meningitis should be covered while waiting, but antibiotics do not investigate a likely inflicted head injury. C is wrong: EEG shows seizure activity but not the cause. E is wrong: electrolytes are worth checking but cannot explain bloody CSF and a bulging fontanelle.

6. E · Strong evidence of sexual abuse; treat, test for other STIs and HIV, assess safety, and report to the police and PSWO

E is correct: gonorrhoea in a prepubertal child beyond the neonatal period is diagnostic of sexual contact unless proven otherwise; treat (ceftriaxone weight-based), screen for HIV, syphilis, chlamydia and HBV, protect and report. A is wrong: non-specific vulvovaginitis does not grow gonococcus. B is wrong: non-sexual transmission via fomites is negligible and must not be assumed. C is wrong: perinatal gonococcal infection presents as neonatal conjunctivitis in the first weeks, and the mother is negative. D is wrong: the culture confirms a genital STI, not a UTI.

7. C · 'I can't promise that, because if someone is hurting you I need to get help to keep you safe; but I will only tell people who can help.'

C is correct: never promise secrecy you cannot keep; explain honestly that you may need to share information to keep the child safe, and only with those who need to know. A is wrong: a broken promise destroys trust, and you cannot keep abuse secret. B is wrong: refusing to listen fails the child and may be the only chance to disclose. D is wrong: the abuser may be a parent; insisting on parental involvement can block disclosure. E is wrong: requiring a parent's presence may silence the child.

8. B · Neglect; involve the social worker and PSWO, support the family and plan follow-up

B is correct: rapid weight gain in hospital with ordinary food, missed immunisations, untreated illness and lack of supervision linked to parental alcohol use point to neglect; this needs a child-protection response with family support. A is wrong: organic causes would not usually allow such rapid catch-up on normal feeds. C is wrong: poverty alone does not explain leaving a 2-year-old alone and missed free services; this is a protection concern, though poverty support may be part of the plan. D is wrong: there is no evidence of inflicted injury; neglect is managed mainly through support and monitoring. E is wrong: WAZ −3.2 is severe underweight, not normal.

9. D · ABC/3TC 600/300 mg one tablet + DTG 50 mg one tablet once daily for 28 days

D is correct: at 26 kg (25–34.9 kg band) a DTG-based paediatric regimen is ABC/3TC 600/300 one tablet + DTG 50 mg once daily for 28 days (UCG 2023 alternatively lists ABC + 3TC + LPV/r). A is wrong: TDF-based TLD is used from 30 kg. B is wrong: single-drug NVP is infant prophylaxis against vertical transmission, not PEP. C is wrong: co-trimoxazole prevents opportunistic infections, not HIV. E is wrong: two drugs for 7 days is an inadequate PEP regimen and duration.

10. A · The anogenital examination should be done once, by the most experienced examiner available, with consent/assent

A is correct: to avoid re-traumatisation and to preserve evidence quality, one examination by the most experienced person, with consent and assent, is best. B is wrong: repeated examinations re-traumatise the child. C is wrong: speculum examination is not done in prepubertal girls except under anaesthesia for injuries. D is wrong: most abused children have normal findings; the hymen can stretch and heal. E is wrong: life-threatening injuries and bleeding are treated first.

11. B · Forced immersion in hot water

B is correct: symmetrical 'stocking' burns with a clear tide-mark, no splash marks and sparing of the soles (pressed against the cooler basin floor) indicate a child held in hot water. A is wrong: spills cause irregular, asymmetrical, arrow-shaped burns with splash marks, usually on the front of the body. C is wrong: contact burns take the shape of the object, not circumferential stockings. D is wrong: chemical burns follow where the substance was applied and do not form a uniform horizontal margin. E is wrong: fire burns are irregular, deep and associated with singed hair or clothing.

12. E · Respect for the views of the child (Article 12)

E is correct: the four general principles are non-discrimination (Art. 2), best interests (Art. 3), life, survival and development (Art. 6), and respect for the child's views (Art. 12). A is wrong: education (Art. 28) is a right but not a general principle. B is wrong: protection from labour (Art. 32) is a protection right, not a general principle. C is wrong: birth registration (Art. 7) is a right, not a principle, and there is no 6-month rule in the Convention. D is wrong: children's duties appear in the African Charter, not among the UNCRC general principles.

13. D · Explain that defilement is a criminal offence that cannot be settled privately, continue PEP, and inform the PSWO and police CFPU so that her safety at home is assessed before discharge

D is correct: aggravated defilement (victim under 14) is a criminal offence that families cannot settle; the child's best interests require continued medical care and a safety assessment before discharge, involving the PSWO and police. A is wrong: 'settling' leaves the child unprotected and is illegal. B is wrong: PEP must be completed for 28 days whatever happens legally. C is wrong: threats escalate conflict and may endanger the child; explain calmly. E is wrong: destroying medical-legal records is unethical and illegal.

14. C · Treat the fever, document the injuries (body map), and report to the PSWO/police; corporal punishment in schools is prohibited and causes harm

C is correct: multiple bruises of different ages from caning are physical abuse; treat, document and report through the protection system (the Children (Amendment) Act 2016 bans corporal punishment in schools; verify the section). A is wrong: cultural acceptance does not make it lawful or harmless. B is wrong: blaming the child ignores the abuse. D is wrong: the clinician's role is to report through proper channels, not to investigate or discipline. E is wrong: moving schools does not protect the other children or address the abuse.

15. A · PEP is no longer recommended (beyond 72 hours), but she still needs STI prophylaxis, hepatitis B vaccination, forensic examination, documentation, psychosocial support, reporting and HIV testing at 6 weeks and 3 months

A is correct: beyond 72 hours PEP is not recommended, but STI prophylaxis, HBV vaccine, examination and evidence (some samples remain useful up to 7 days), support, reporting and follow-up HIV tests remain essential. B is wrong: PEP efficacy beyond 72 hours is unproven and it is not recommended. C is wrong: she is 7 years old and premenarchal, so emergency contraception is not indicated. D is wrong: there is still much medical and psychosocial care to give. E is wrong: follow-up HIV testing is at 6 weeks and 3 months, not 2 years.

SELF-TEST

Clinical cases

Work through each step before reading the model answer.

Case 1: The baby with a 'fall from the bed'

At 21:00 a 4-month-old boy (5.8 kg, WAZ −0.6) is brought to Mengo casualty by his mother's boyfriend, who says the baby 'fell off the bed this afternoon and has been crying'. The baby is irritable, with a swollen right arm, a 2 cm bruise behind the left ear and small bruises on the chest. He is fully immunised for age and exclusively breastfed. The mother is at work and cannot be reached.

1. What features of this history and examination raise concern?

> **Non-mobile infant** (4 months) with bruises in **TEN-4** sites (ear, torso); bruising in any infant under 4 months is a red flag; a **short fall** rarely causes a fracture; brought by a **non-parent** partner (risk factor); possible **delay**.

2. What are your immediate actions?

> ABC; **analgesia** (paracetamol 15 mg/kg ≈ 90 mg); immobilise the arm; **admit** (place of safety); inform the **senior on call**; document the history **verbatim** (who said what); body map and measured descriptions; contact the mother.

3. What investigations will you request and why?

> **X-ray of the arm**; **skeletal survey** (occult rib/metaphyseal fractures); **CT head** (under 1 year: occult subdural); **fundoscopy**; **FBC, platelets, clotting** (bleeding disorder mimic); bone profile (Ca, PO_4, ALP).

4. The skeletal survey shows a transverse right humeral fracture, two healing posterior rib fractures and a metaphyseal corner fracture of the left tibia. Clotting is normal. Interpret.

> **Multiple fractures of different ages**, including **posterior rib** and **metaphyseal** fractures that are highly specific for **inflicted injury** (squeezing and shaking/twisting). Normal clotting excludes a bleeding mimic. This is **physical abuse** until proven otherwise.

5. Who do you involve and what do you document?

> **Medical social worker**, district **PSWO** and police **CFPU** (same day); the **PF3** when issued; a multidisciplinary meeting. Document clearly and factually: injuries, dates, the explanation given, and that the findings are **not consistent** with a fall from a bed. Assess **other children** in the household.

6. The mother arrives and is very distressed. How do you speak to her?

> Privately, calmly and **non-judgementally**: explain the injuries, that they are not explained by the history, that the law requires you to involve the PSWO and police to keep the baby safe, and what happens next. Listen to her account (she may also be a victim of domestic violence). Do not accuse anyone; avoid speculation.

Take-away: Any bruise or fracture in a non-mobile infant demands admission, a skeletal survey, CT head and clotting, careful documentation and same-day referral to the PSWO and police; posterior rib and metaphyseal fractures are close to diagnostic of abuse.

Case 2: A child defiled: the first 72 hours

A 6-year-old girl (19 kg) is brought by her grandmother to a HC IV in Mukono at 10:00. Last night at about 20:00 a 30-year-old male tenant 'took her into his room'. She has pain on passing urine and blood on her underwear. The grandmother has kept the underwear in a plastic bag. HIV status of the girl: negative (tested

at 18 months); of the tenant: unknown.

1. What is the legal classification, and what are your time-critical medical priorities?

> **Aggravated defilement** (victim under 14). About **14 hours** have passed: **HIV PEP** (start now), **forensic evidence** (before washing; underwear moved into a **paper** bag), **injury assessment**, **STI prophylaxis**, **HepB vaccine** check. EC is not indicated (premenarchal).

2. How will you take the history and obtain consent?

> From the grandmother, then gently from the child with the grandmother present if she wishes; **open questions**, record the child's **exact words**, avoid repeated questioning. Explain each step; obtain **consent** from the grandmother and **assent** from the child; the child can say 'stop'.

3. Examination shows a 1 cm fresh laceration of the posterior fourchette with slight bleeding, and bruising of the labia minora. What do you do?

> Collect **swabs** (vulval/vaginal introitus) with chain of custody; control bleeding; if the laceration is deep or bleeding persists, refer for **examination under anaesthesia** and repair. Analgesia (paracetamol 15 mg/kg ≈ 285 mg). Document on a body map.

4. Write her prophylaxis.

> **PEP 28 days** (14–19.9 kg band): **ABC/3TC 120/60 mg 2.5 tablets + pDTG 10 mg 2.5 tablets** once daily (or ABC + 3TC + LPV/r per UCG 2023 if DTG unavailable). **STI prophylaxis**: ceftriaxone **50 mg/kg ≈ 250 mg IM** single dose (max 250 mg; verify paediatric dose), **azithromycin 20 mg/kg ≈ 380 mg** single dose, metronidazole weight-based. **Hepatitis B vaccine** if not fully immunised (she had pentavalent: check card). Baseline **HIV, syphilis, HBsAg**.

5. What reporting and safety steps follow?

> Report to the **police CFPU** (PF3A) and the **PSWO**; ensure the tenant has **no access** to the child (safe placement with grandmother if protective); involve the **medical social worker**; Sauti 116 if advice is needed.

6. What follow-up does she need?

> PEP adherence and side-effect check at **2 weeks**; HIV tests at **6 weeks and 3 months**; syphilis repeat; wound review; **psychosocial counselling** (trauma-focused) for her and the grandmother; school support; preparation for court (the clinician may be a witness).

> **Take-away:** In child sexual abuse the first hours count: start PEP and STI prophylaxis, preserve evidence, examine once gently, document the child's words, report to police and PSWO, and keep her safe; long-term psychosocial support matters as much as the drugs.

Case 3: The thin toddler: neglect or poverty?

A 22-month-old girl from a Kampala informal settlement is admitted with diarrhoea. Weight 7.1 kg (WAZ −3.6), length 76 cm (LAZ −2.4), MUAC 11.3 cm, no oedema. Her card shows only BCG and 6-week vaccines. Her 17-year-old mother works long hours selling food at a taxi park and leaves her with a 7-year-old sibling. The mother is HIV-positive; the child's HIV status is unknown.

1. What are the medical priorities?

> **SAM** (MUAC <11.5 cm): manage per WHO 2023 SAM guidance: careful rehydration (ReSoMal, not Plan C unless shocked), treat hypoglycaemia/hypothermia, antibiotics, F-75 then F-100/RUTF, micronutrients. **HIV rapid test** (valid at 22 months if she stopped breastfeeding more than 6 weeks ago; otherwise DNA PCR). TB screen.

2. Her HIV test is positive. What does this add?

> She has HIV with SAM (WHO stage 4 if not responding to treatment). Start CTX, screen for TB, start **ART within 2 weeks** once stable. This also reveals a **missed EID** (system failure, possible neglect of care).

3. Is this neglect, poverty, or both? How will you decide?

> Explore whether the mother can access and use resources: did she know about EID, immunisation and free services? Was she supported? A **17-year-old mother** working to survive is herself a **child** with rights. This is largely **poverty and lack of support** with elements of **neglect** (no supervision, missed care). Decide with the social worker after assessment, not on first impressions.

4. Note: the mother is 17. What protection issues arise for her?

> She became pregnant at about 15: **defilement** occurred. Assess her safety, her own HIV care and viral load, contraception, and whether she is in school; she needs **adolescent-friendly HIV services** and social protection. Involve the PSWO for both mother and child.

5. What is your discharge plan?

> OTP/RUTF follow-up weekly, ART clinic for both (same day appointments), **catch-up immunisation**, link to an **OVC/social protection** programme and childcare support, VHT home visits, family planning for the mother, PSWO follow-up to check supervision arrangements.

Take-away: Distinguishing neglect from poverty needs assessment, not assumption; the right response is usually support and monitoring, and a young mother may herself be a child whose rights have been violated.

References

1. Child maltreatment: fact sheet. *World Health Organization*. https://www.who.int/news-room/fact-sheets/detail/child-maltreatment
2. Children's version of the Convention on the Rights of the Child. *UNICEF*. https://www.unicef.org/child-rights-convention/convention-text-childrens-version
3. Child maltreatment: when to suspect maltreatment in under 18s (CG89). *National Institute for Health and Care Excellence*, 2017. https://www.nice.org.uk/guidance/cg89
4. Child abuse: clinical practice guideline. *Royal Children's Hospital Melbourne*. https://www.rch.org.au/clinicalguide/guideline_index/Child_abuse_guideline/
5. Overview of Child Maltreatment. *MSD Manual Professional Edition*. https://www.msdmanuals.com/professional/pediatrics/child-maltreatment/overview-of-child-maltreatment
6. Uganda Clinical Guidelines 2023 (Section 1.2.6: Sexual assault/rape). *Ministry of Health Uganda*, 2023. https://library.health.go.ug/sites/default/files/resources/Uganda%20Clinical%20Guidelines%202023.pdf
7. Consolidated Guidelines for the Prevention and Treatment of HIV and AIDS in Uganda (PEP). *Ministry of Health Uganda*, 2022. https://dsduganda.com/wp-content/uploads/2023/05/Consolidated-HIV-and-AIDS-Guidelines-20230516.pdf

A study aid, not a protocol. Check doses against the Uganda Clinical Guidelines 2023, your unit's protocol and a senior before you treat a patient.